

Table 1: HIV service delivery recommendations

Date*	Recommendations	Status
Linkage and ART initiation		
2016	Following an HIV diagnosis, a package of support interventions should be offered to ensure timely linkage to care for all people living with HIV (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
2021	Health systems should invest in people-centred practices and communication, including ongoing training, mentoring, supportive supervision and monitoring of health workers, to improve the relationships between clients and health-care providers (<i>Good practice statement</i>).	Revalidated
2019	ART initiation should follow the overarching principles of providing people-centred care. People-centred care should be focused and organized around the health needs, preferences and expectations of people and communities, upholding individual dignity and respect, especially for vulnerable populations (<i>Good practice statement</i>).	Revalidated
2021	Antiretroviral therapy (ART) initiation may be offered outside the health facility (<i>Conditional recommendation, low- to moderate-certainty evidence</i>).	Revalidated
2017	Rapid ART initiation should be offered to all people living with HIV following a confirmed HIV diagnosis and clinical assessment (<i>Strong recommendation: high-certainty evidence for adults, low-certainty evidence for children</i>).	Revalidated
2021	The offer of same-day ART initiation should include approaches to improve uptake, treatment adherence and retention such as tailored client education, counselling and support (<i>Good practice statement</i>).	Revalidated
Clinical visits and medication pick-up		
2021	People established on ART should be offered clinical visits every 3–6 months, preferably every six months if feasible (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
2021	People established on ART should be offered refills of ART lasting 3–6 months, preferably six months if feasible (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
Adherence and engagement in care		
2025	Adherence support interventions should be provided to people on ART (<i>Strong recommendation, moderate-certainty evidence</i>).	Updated
2021	Viral load for treatment monitoring should be complemented with non-judgemental, tailored approaches to assessing adherence (<i>Good practice statement</i>).	Revalidated
2021	HIV programmes should implement interventions to trace people who have disengaged from care and provide support for re-engagement (<i>Strong recommendation, low-certainty evidence</i>).	Revalidated
Services for adolescents and young people		
2021	Psychosocial interventions should be provided to all adolescents and young adults living with HIV (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
2013	Adolescent-friendly services should be implemented in HIV services to ensure engagement and improved outcomes (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated

Table 1: HIV service delivery recommendations (cont'd)

Date*	Recommendations	Status
Services for adolescents and young people		
2013	Adolescents should be counselled about the potential benefits and risks of disclosing their HIV status to others and empowered and supported to determine whether, when, how and to whom to disclose (<i>Conditional recommendation, very-low-certainty evidence</i>).	Revalidated
Task sharing		
2016	Trained non-physician clinicians, midwives and nurses can initiate first-line ART (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
2016	Trained non-physician clinicians, midwives and nurses can maintain ART (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
2016	Trained and supervised community health workers can dispense ART between regular clinical visits (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
2016	Trained and supervised lay health-care providers can distribute ART (<i>Strong recommendation, low-certainty evidence</i>).	Revalidated
2021	Task sharing of specimen collection and point-of-care testing with non-laboratory personnel should be implemented when professional staffing capacity is limited (<i>Strong recommendation, moderate-certainty evidence</i>).	Revalidated
Service Delivery models and integration		
2013	Decentralization of ART care should be considered as a way to increase access and improve retention in care (<i>Strong recommendation, moderate-to-low-certainty evidence</i>).	Revalidated
2013	In generalized epidemic settings, ART should be initiated and maintained in pregnant and postpartum women and in infants at maternal and child health-care settings, with linkage and referral to ongoing HIV care and ART, if appropriate (<i>Strong recommendation, low-certainty evidence</i>).	Revalidated
2021	Sexual and reproductive health services, including contraception, may be integrated with HIV services (<i>Conditional recommendation, very-low-certainty evidence</i>).	Revalidated
2013	In settings with a high burden of HIV and tuberculosis (TB), ART should be initiated in TB treatment settings, with linkage to ongoing HIV care and ART (<i>Strong recommendation, very-low-certainty evidence</i>).	Revalidated
2013	In settings with a high burden of HIV and TB, TB treatment may be provided for people living with HIV in HIV care settings if they have been diagnosed with TB (<i>Strong recommendation, very-low-certainty evidence</i>).	Revalidated
2025	Diabetes and hypertension care should be integrated with HIV services (<i>Strong recommendation, moderate-to-very-low-certainty evidence</i>).	Updated
2025	Mental health care for depression, anxiety and alcohol use disorders should be integrated with HIV services (<i>Strong recommendation, moderate-to-very-low-certainty evidence</i>).	New

* Date the recommendation/good practice statement was formulated

1. Introduction

HIV remains a major public health challenge and a leading cause of illness and death in many low- and middle-income countries, despite the wide availability of effective antiretroviral therapy and proven prevention tools.

In many settings, evidence-informed interventions for HIV prevention, testing, treatment and long-term care are not fully implemented, delivered inconsistently or reach people too late, contributing to ongoing transmission, advanced disease and avoidable mortality. Persistent health system constraints – including workforce shortages, weak supply chains and limited integration with related services such as those for noncommunicable diseases and mental health care – further hinder effective service delivery.

These HIV service delivery guidelines provide updated and consolidated recommendations to support countries, programme managers, health-care workers, communities and other stakeholders to organize services that are people-centred, integrated, equitable and grounded in the best available evidence. The guidelines aim to improve access to and continuity of high-quality HIV services, reduce inequities in outcomes and accelerate progress towards ending AIDS as a public health threat.

2. Linkage and ART initiation

2.1 Linkage from HIV testing to enrolment in care

Recommendation

Following an HIV diagnosis, a package of support interventions should be offered to ensure timely linkage to care for all people living with HIV.

(Strong recommendation, moderate-certainty evidence)

The following interventions have demonstrated benefit in improving linkage to care following an HIV diagnosis:

- streamlined interventions to reduce time between diagnosis and engagement in care, including (i) enhanced linkage with case management; (ii) support for HIV disclosure; (iii) tracing; (iv) training staff to provide multiple services; and (v) streamlined services (*moderate-certainty evidence*);
- peer support^a and navigation approaches for linkage (*moderate-certainty evidence*); and
- quality improvement approaches using data to improve linkage (*low-certainty evidence*).

^a Includes peer counselling.

Revalidated

Good practice statements

ART initiation should follow the overarching principles of providing people-centred care.

People-centred care is care that is focused and organized around the health needs, preferences and expectations of people and communities, upholding individual dignity and respect, especially for vulnerable populations. It should promote the engagement and support of people and families to play an active role in their own care through informed decision-making.

All people newly diagnosed with HIV should be retested to verify their HIV status before starting ART, using the same testing strategy and algorithm as the initial test. To minimize the risk of misdiagnosis, this approach should be maintained in settings in which rapid ART initiation is being implemented.

Revalidated

Background

Linkage to HIV prevention, treatment and care after testing remains a challenge, with disproportionate gaps for key populations, men, young people and previously diagnosed people living with HIV (not receiving ART, disengaged or lost to care).

HIV testing service providers are primarily responsible for linkage to care. People testing HIV positive should be assessed for advanced HIV disease using CD4 cell count testing and offered ART, and those testing negative should be linked to prevention services.

Rationale

Evidence-informed interventions to support linkage can be classified into three approaches (2):

- streamlined services via enhanced linkage with case management, HIV disclosure support, client tracing for non-engagers and staff training for multiple services;
- peer support and navigation interventions, including home visits, system navigation and enhanced counselling; and
- quality improvement interventions enhance linkage to care (3).

Implementation considerations

- Interventions may vary based on the context, including health-care delivery systems, geography and target population.
- A combination of interventions may be needed to improve linkage for specific underserved groups, especially key populations and men.
- Barriers to linkage to HIV care and ART initiation should be assessed and addressed, especially for populations facing multiple structural and individual barriers.
- Linkage for children can be improved using point-of-care early infant diagnosis, SMS/GSM/GPRS printers to accelerate access to laboratory results and family-centred models delivering care to mother, baby, and partner at the same point.
- Health facilities should have capacity to provide services for key populations. Community- and peer-based approaches can be especially effective in restrictive legal settings.

WHO guidelines on HIV testing services (4) provide information on post-test services and linkage to care.

2.2 People-centred care

Good practice statement

Health systems should invest in people-centred practices and communication, including ongoing training, mentoring, supportive supervision and monitoring of health workers, to improve the relationships between clients and health-care providers.

HIV programmes should:

- provide people-centred care that is focused and organized around the health needs, preferences and expectations of people and communities, upholding individual dignity and respect, especially for underserved populations, and engage and support people and families to play an active role in their own care by informed decision-making;
- offer safe, acceptable and appropriate clinical and non-clinical services in a timely fashion, aiming to reduce morbidity and mortality associated with HIV infection and to improve health outcomes and quality of life in general; and
- promote the efficient and effective use of resources.

Revalidated

Background

People-centred health services are an approach to care that consciously adopts the perspectives of individuals, families and communities and sees them as participants and beneficiaries of trusted health systems that respond to their needs and preferences in humane and holistic ways. This approach acknowledges the experiences and perspectives of health-care providers that may enable or prevent the delivery of high-quality people-centred care.

Rationale

- A systematic review (5) identified several interventions to enhance people-centred care for people living with HIV. Overall, studies reported beneficial effects of these approaches across the HIV cascade outcomes, including improved ART uptake, adherence and viral load suppression.
- Studies have shown that people are willing to travel longer distances to be seen by a health-care provider with a respectful and caring attitude, and negative health-care worker attitudes contribute to loss to care and poor programme outcomes (6).

Interventions found to improve relationships between clients and health-care providers can be classified into the following approaches:

- providing friendly and welcoming services: training providers to make general HIV services more welcoming, including for clients who have disengaged from care;
- conducting sensitization training for clinical and non-clinical health-care providers to improve care for key populations both at the primary care and community levels; such training should include issues related to stigma and discrimination;
- offering individualized adherence counselling and person-centred communication such as shared decision-making and planning for ART initiation and adherence;
- facilitating client education in empowerment and communication skills; and
- providing feedback to health-care workers on client concerns and evaluation of service quality such as community score cards and client feedback mechanisms combined with quality improvement exercises.

Implementation considerations

- Build capacity for ongoing training, mentoring and supportive supervision on people-centred communication, stigma reduction and welcoming services.
- Integrate sensitization training for all staff (clinical and non-clinical) on respectful attitudes, stigma and discrimination, re-engagement of clients who have disengaged, and support for women and other vulnerable groups experiencing violence or sexual health concerns.
- Establish routine client feedback mechanisms (such as satisfaction questionnaires, community score cards and suggestion boxes) and use the results in facility quality-improvement meetings.
- Involve clients and community representatives in designing and reviewing services.
- Review workload, staffing and client flow to allow time for person-centred interactions.